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Name: هبه حسني مليجي محمد National ID: 27806132102003	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
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تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
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المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Supportive and palliative Treatment
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التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



DMS
مفتاح





وزارة الصحة
مملكة العربية السعودية
مركز صحة المرأة
MDT FORM 2/2/2028-6

الطبيب العام
MDT FORM 2/2/2028-6

MDT FORM 2/2/2028-6

Name	د. محمد	MRN	CCCA11010
Age	44 years	ID	CNA-11010-1
Marital status	Married	Phone	01148503379
NRS SCORE		Marital status	CNA-11010-1
BMI		Family history	
Comorbidity		Reason of MDT	PC

Sex: Female Clinical Date: 20/1/2028 OS 3

ECG: 2/18 3/18 4/2 5/2 6/2 7/2 8/2 9/2 10/2 11/2 12/2

Sonogrammetry:

Mechanics: Cancer history I V received

Mets Workup:

VC Metastasis - WJ T-Ad T-Ad

WJ CT 18/1/2025 - 9 PD regional

WJ CT 18/1/2025 - 9 PD regional

Recommendations:

20/1/2028

MDT OUTCOME & PLAN

MDT Decision: Chemotherapy
Signature: Dr. Mohamed Al-Harbi

MDT FORM 27 / 9 / 2025 G

Name	د. محمد م. م. م.	MRN	0000 / 0000
Age	47y	ID	
Menopausal status	Premy. D	Phone	0114 8503337
NRS SCORE		Marital status	CVN-7134100
BMI		Family history	
CO morbidity		REASON OF MDT	New

Clinical Data

BX: I.D.C. II				R 20%	PS: 1
ER: tu 7/8	PR: tu 8/8	HER 2: 2+	Ki67: SISH: Ampl		
Sonomammography:					
Mets Workup:				CT 2: N1: HX CT on 18/7/2020 2 HFL. IVb VI m	

Recommendations:	+ Left axilla CT on 18/7/2020 Left lower chest may + pi 3x2 BS AT phase CT: pancy
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MDT OUTCOME & PLAN	for Tazadweek + Heroplin IV 9/10
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SIGNATURE					
MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric



ImmunohistoChemistry report

Report Number: m.1105

Female	النوع	هبة حسنى مليجي محمد	م المريض
تقارير	العنوان	47 Y	سن
23/07/2025 12:58	تاريخ استلام العينة		رابع اخذ العينة
30/07/2025 13:27	تاريخ التقرير	مركز صحة المرأة	التيه الدخيل
		مركز صحة المرأة	مباية

Primary Sample :

Paraffin Block

Test :

HER2 by SISH

Comment :

Technique :

sample received was stained for HER2 gene and chromosome 17 using INFORM HER2 Dual Probe Cocktail technique in benchmark ULTRA (VENTANA).

Result :

The ratio of Her 2 / chr 17 is more than 2

Referral lab :

Comment :

Conclusion :

Her 2 Status by SISH (Dual Ish Technique): Positive (amplified Her 2 / chr 17 > 2) (CORE BIOPSY)

Pathologist:

د. هبة حسنى مليجي محمد

Lab Director:

د. هبة حسنى مليجي محمد



Patient ID: 26178	Patient Name English: Heba Hassan Meigi
مرضى 1 شهر	2025/07/17 10:15 26 AM Report Date

Women's Health Outreach Program

Name: Heba Hassan

Date: 17/07/2025

Bilateral Full field Digital Mammography and high-resolution ultrasound.

Clinical summary: 47 years old female patient with left breast lump.

Detailed Findings

Left breast:

- (ACR density C).
- The Left breast shows deeply seated posterior 1/3 large ill-defined spiculated dense mass lesion with obscured margins and architectural distortion and abutting the underlying muscles and located at lower central region.
- Ultrasound revealed large ill-defined irregular malignant hypoechoic mass lesion measures about 3.3 x 2.3 cm at its maximum diameters and located at 6 ° clock position (Zone C). It is seen no line of cleavage with the underlying muscle suggestive of muscle invasion.
- Normal nipple/areolar complex (inverted nipple).
- Multiple enlarged axillary lymph nodes at level I & II with thickened cortex, largest one measures about 24 x 11 mm at level I with focal cortical thickening measures about 7.5 mm.

Right breast:

- (ACR density C).
- No masses, asymmetric densities, distortion or suspicious clusters of micro calcifications.
- Ultrasound demonstrated small well-defined hypoechoic mass lesion measures about 8.1 x 4.5 mm and located at para-areolar 12 ° clock position.
- Normal skin/subcutaneous tissue and nipple/areolar complex.
- Non-specific axillary LNs.

Impression:

- Left breast (BiRADS 5): Left lower central malignant mass lesion with ipsilateral pathological axillary lymph nodes as described. Tru cut biopsy was done on the same setting for dynamic MRI breast study.
- Right breast (BiRADS 3): Right small upper central probably benign mass lesion.

Much obliged,
Dr. Hedayat Nour, MD



Patient Name: Heba Hosny Meligy

Date : 18/08/2025

Exam : CT CAP+C (CT Chest Abd. Pelvis)

Age : 47 Year(s)

Medical ID:

Pat. ID: 110996

POST-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND PELVIS: -**Findings:****Reported case of right breast cancer****I Chest:-**

- Left breast mass at LIQ measuring 2.5 x2.1 cm with bilateral inverted nipples.
- Few left axillary lymph nodes , the largest 19 x10 mm likely reactive.
- No sizable mediastinal L. Ns .
- Clear both lung fields. No sizable pulmonary nodules, masses, areas of consolidation or cavitary lesions.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

II Abdomen & pelvis:

- The liver is mildly enlarged with homogenous fatty CT texture. two focal lesions , one seen at segment IV b measures 24 x 14 mm showing faint arterial enhancement, the other seen at segment VI measuring 33 x 29 mm showing arterial peripheral nodular enhancement and become isodense in delayed phase
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Average size gall bladder with hyperdense stone 4mm.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.
- Few osteolytic lesions of both iliac bones .

Opinion:

- Left breast mass.
- Two hepatic focal lesion , one in segment VI likely hemangioma , segment IV focal lesion likely metastatic for triphasic study.
- Calculus gall bladder .
- Few osteolytic lesions of both iliac bones for correlation with bone scan.